



“Gaudium IVF and Women Health Limited
Q4 & FY26 Earnings Conference Call”

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**MANAGEMENT: DR. MANIKA KHANNA – CHAIRPERSON AND
MANAGING DIRECTOR – GAUDIUM IVF AND WOMEN
HEALTH LIMITED
MR. RAKESH SHARMA – CHIEF FINANCIAL OFFICER
– GAUDIUM IVF AND WOMEN HEALTH LIMITED
MR. VISHAD KHANNA – DIRECTOR – GAUDIUM IVF
AND WOMEN HEALTH LIMITED**

Moderator: Ladies and gentlemen, good day and welcome to the Gaudium IVF and Women Health Limited Q4 and FY26 Earnings Conference Call. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions, and expectations of the company as on the date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Dr. Manika Khanna, Chairperson and Managing Director of Gaudium IVF and Women Health Limited. Thank you and over to you, ma'am.

Manika Khanna: Thank you, Rutuja.

Manika Khanna: On behalf of Gaudium IVF and Women Health Limited, I welcome you all to our Q4 FY26 and full year 2026 Earnings Conference Call. As you know, we recently became the country's first IVF company to be listed on the Mainboard. This is our maiden full year results as a publicly listed company.

With me on the call today is Mr. Rakesh Sharma, our Chief Financial Officer, who will take you through the financials, and Mr. Vishad Khanna, our Director. We are also supported by our Investor Relations Partners, Adfactors PR.

Now, you know it's very important because we are not in a traditional business. We are in the business of building, actually creating lives and the business of hope. So, before I speak about the business, I would like to share a little about the philosophy that built Gaudium because our institution is deeply rooted in clinical excellence and patient trust.

Well, that starts with me because I am an MD in Obstetrics and Gynaecology with advanced training in gynaecological endoscopic surgery and IVF training at Germany and Melbourne. Over the last 16 years, I have dedicated my life to reproductive medicine.

My entrepreneurial journey transitioned after 2017-'18, I would say. From 2009 to 2017 was a doctor's journey seeking excellence, and post that was the entrepreneurial transition when we just started our expansion. I always remain a doctor first. Gaudium was never created as a financial platform entering healthcare. It was built from real-world clinical practice.

We grew ground up, and while growing, we have learned a lot, and this learning curve is our

biggest strength going forward. Every protocol we follow, every technology we invest in, every doctor we train, and every patient journey we design, is led by clinical understanding and outcome-oriented thinking. We are a very success-driven company.

So, in IVF, success matters a lot, and we keep a patient-first policy and a success-driven approach at the heart of everything. The more babies we make, the more business we make, and the more business we invite in the future.

Infertility care success is not measured in just transactions. It is actually building a family. You are creating life, fulfilling dreams of thousands of mothers who want to have a baby. Deep clinical leadership is not just an advantage; it is the foundation of long-term trust, and, in fact, this is our biggest sustainable differentiator in this industry because the leader is a clinician.

At Gaudium, our USP, as I said, is clinical excellence. It is not separate from the management, and it drives the management. I believe that this clinician-led DNA will continue to remain one of our strongest and most enduring competitive advantages as we scale the platform further.

Now, let me just give you an overview of the industry. For the first time in the history of our country, the fertility rate has fallen down below the replacement level. Which means that many children are not being born as are needed to replace the population. It is very important that we pay attention to this.

That means infertility is also rising. 15% to 20% of 1.5 billion people of us are infertile. You can just imagine the numbers, and right now out of that, only 2% are coming forward. So, there is this huge untapped market that is lying there, which can be tapped by becoming more accessible, affordable, and that is what Gaudium's vision is working on.

The Indian IVF market today stands at approximately USD 1.32 billion and is projected to grow to USD 4.54 billion by 2034 at a CAGR of about 13%. India's share of the global IVF market is right now around 4.8%, but it will rise up to 8.3% by 2034. That is significant.

Behind those numbers lies a more important reality, which I just said. An estimated 27.5 million couples in India are affected by infertility, but only 3 lakh IVF cycles are being performed. You can understand how tremendous the gap is. Add to this that people are marrying late, girls are planning to become mothers late.

Awareness is improving thanks to the media. In fact, I have seen a few changes since we became listed. During the IPO roadshows, there was a lot of knowledge seeking by everyone on what IVF is, how it works. Growing awareness is helping. The stigma is definitely reducing. Initially, way back when I started in 2009, even the husband would not think that he needed to be tested.

When we would tell him to get tested, he would just say that he is actually fine. At least now

when they come, they both know that they have to be tested. This is a small change, but it shows that the stigma is reducing. People are accepting the treatment more and more.

The ART and Surrogacy Acts of 2021 have brought regularity and clarity to this industry, which was very important. It is highly beneficial to organized players like Gaudium because when the industry becomes regulated, only good, ethical players with high clinical standards will survive the impact of the regulation.

And it will lead to more and more consolidation in the industry as we move forward. Right now, almost 70% is standalone clinics and unorganized. And so, 30% is the organized and this is going to change very soon. There is also a very meaningful medical tourism opportunity because the cost of an IVF cycle in India is roughly one-fifth of what it is in the United States.

Gaudium is already serving patients from more than 30 countries across the world. We are very proud of that. We are able to do that because we have international accreditations, awards, and high clinical standards. An international patient doesn't just turn up to your doorstep; they do a lot of research before they come. We are happy that we are serving more than 30 countries here.

So, just to summarize, we are operating in a category that is large, underpenetrated, and structurally tailwind supported. Gaudium is best positioned to be one of the principal beneficiaries of that shift.

Now coming to our performance, FY26 has been a defining year for Gaudium IVF. The year we transitioned into a listed, institutional, and scalable fertility platform. Our IPO closed on the 24th of February 2026 and listed on the 27th of February, raising approximately INR 165 crore.

For the full year, our revenue, EBITDA, and PAT have all grown meaningfully over FY25, and Rakesh, our CFO, will walk you through the precise numbers in a few minutes. On our three business segments, IVF treatment remains the engine of the business, and it will always remain the engine of the business.

It contributed about 79% of our revenue in FY25, and we expect this share to expand further as we scale. So, now we have built a lot of muscle. We took a lot of time to build the right foundation. We are now at a cut-copy-paste kind of stage where an SOP-driven model has been tested across the North, South, West, and East of India, and we now just have to increase our accessibility, open more number of hubs and spokes, and the business is only going to grow.

We have completed over 50,000 successful treatments in infertility since inception. Within IVF, we offer the full spectrum of services, be it a natural cycle, an IUI, an IVF, ICSI, down to advanced procedures like egg and embryo freezing, sperm freezing, pre-implantation genetic testing.

We are very proud that in male infertility treatments, we are one of the most advanced players. Egg freezing is very close to our hearts because I always feel that our young daughters, who are working so hard in their careers, studying so hard in school and college, should have the freedom to preserve their eggs in time so as to preserve their fertility and not put a brake to their careers.

Egg freezing, I call it reproductive freedom. I've called it that at the Forbes platform, I've called it at various podcasts. Recently I spoke about it at the Forbes platform, and it invoked a lot of curiosity. It is very important and is a huge business area for growth.

Our 15-bed hospital facility at Janakpuri rounds off the patient journey from fertility treatment through delivery and postnatal care. A continuum that very few peers in the listed space offer today. On pharmacy, I want to address this directly. It is a small part of our business today and is going to remain a small part.

It is not a segment we intend to scale with specific focus as an independent growth vertical. Its role is only to support the IVF patient journey, ensuring consistent availability of specialized medications and consumables. We are very results-conscious, so we want the right medicines reaching our patients, and hence our pharmacy wing allows us to do that.

Capital allocation, management attention, and growth investments will be directed predominantly towards the IVF treatments and expansion of our hub and spoke network. Now if there is one philosophy that runs through everything we do at Gaudium, it is continuous innovation. That is what sets us apart from many peers.

We have done a lot of firsts. We are the first to get a European Society Quality Research Award. We are the first to be listed on the stock exchange. We are the first to have brought in AI, which I'll be just throwing some light on AI tools, SiD and ERICA, into India.

We were among the early adopters of vitrification. Vitrification, earlier egg and embryo freezing used to be done by a slow method. Vitrification revolutionized this. We were amongst the first early adopters of vitrification, egg freezing, pre-implantation genetic testing, and TESA, that is Testicular Aspiration, where the semen count is close to zero, is one of our specialties.

So, on the 1st of April 2026, just a few weeks after our listing, we became the first IVF chain in India to formally integrate AI-led embryology into routine clinical practices in partnership with IVF 2.0, a UK headquartered Embryology Innovation Organization. This organization has been founded by globally renowned embryologists, Dr. Jacques Cohen and Dr. Alejandro Chavez.

The integration was formally inaugurated by the Honourable Union Minister Shri Nitin Gadkari very recently. What this means in practice is the deployment of two AI systems into our clinical workflow. SiD and ERICA. Now very in a very layman kind of term I would like to explain what SiD does. SiD helps us identify the best quality sperm to be chosen while

doing a live ICSI. It is beautiful. It marks, it becomes like an AI assistant to the embryologist, and it helps him choose the right sperm, the best sperm.

And then once the embryo is made, ERICA is the embryo ranking intelligent classification assistant. This is again a beautiful assistant because it studies 2.5 million parameters on the health of the embryo. It selects the best, the most likely to be healthy embryo, not just genetically but metabolically.

And hence both these tools when used definitely increase the success rate. And most importantly, they increase the success rate at first attempt. So, it reduces the number of attempts the patient would require to get pregnant because every repeat cycle is avoided, which is a reduction in emotional, physical, and financial burden carried by a patient. And any new technology that we adopt, we keep the patient-first policy in mind, and we see how much value it adds for the patient and for us business-wise.

Looking ahead, our priority is execution of the 19 new IVF hubs planned under our IPO use of proceeds, three hubs are expected to be operational within the next couple of months. As we speak, the first very big hub we are opening in South of Delhi will be inaugurated in the coming month. And two others are in progress in Nagpur and in Gurgaon.

And with this, we will also be adding nine spokes. These are all aimed in the next couple of months. And I am very happy to share that, looking at the response we are getting from medical tourism, we are adding three international spokes to rope in more patients in Nigeria, Sydney, and Paris.

The balance of Phase 1 will follow, and in totality, 10 new hubs in FY27, 8 in FY28, and 1 in FY29. This phased rollout takes us deeper into Tier 2 markets while reinforcing our existing metro hubs and with additional spokes. Our asset-light hub and spoke model has been refined over 16 years and is, in our view, one of the most capital-efficient ways to scale fertility care in India, which reflects in our EBITDA and PAT as you will soon see.

Let me close with this: the Indian fertility market is large, underpenetrated, and going through a once-in-a-generation institutionalization. Gaudium has spent 16 years building the clinical foundation, the brand, and the operating model required to lead this transition.

With a debt-light balance sheet post IPO, almost zero, a clearly funded expansion plan, an innovation-first culture, clinical excellence at the core, and a category that is just beginning its growth curve in India, we believe Gaudium IVF is well placed to compound growth responsibly over the coming years.

With that, let me hand over to Rakesh, our CFO, to take you through the financials. Thank you.

Rakesh Sharma:

Thank you, Doctor. Good afternoon, everyone. Let me take you through the financial statement and the financial performance for FY26. Starting with the quarter-on-quarter

results, comparative Q4 of FY26 and '25. The revenue from operations stood at INR 30.35 crore against INR 26.92 crore in Q4 FY25, a year-on-year growth of 12.74%.

On a sequential basis, this compares with INR 24.5 crore in Q3 FY26, a quarter-on-quarter growth of 23.85%. Growth was driven by higher volume at mature hubs and growing demand. EBITDA for the quarter was INR 12.17 crore against INR 8.72 crore in Q4 FY25, a growth of 39.56% with an EBITDA margin of 40.10%.

PAT for the quarter was at INR 8.36 crore against INR 6.45 crore in Q4 FY25, a growth of 29.46%. PAT margin of 27.54%. Sequentially, this compares with 3.63 crore in Q3 FY26. For the full year, the revenue stood at INR 104.35 crore against INR 70.72 crore in FY25, a growth of 47.56%. EBITDA was INR 37.7 crore against INR 28.63 crore in FY25, a growth of 31.69%. PAT from continuing operations was INR 24.48 crore against INR 19.17 crore in FY25.

On the capex, during FY26, the company incurred a capital expenditure of approximately INR2.11 crore. For FY27, we expect a capex of INR 25 crore, which will be due to the new hubs at an estimated average cost of INR 2.5 crores per hub, funded primarily from IPO proceeds and internal accruals.

Cash and bank together stood at approximately INR 8.78 crore as of 31st March 2026, against INR 7.21 crore at FY25. On material strengthening, total borrowings stood at approximately INR 24.56 crore against INR 20.61 crore. Our consolidated debt-to-equity ratio at the year-end stood at approximately 0.16 times against 0.45 times at the end of FY25. The company today operates with a debt-light balance sheet that allows us to fund planned expansion entirely through internal resources.

To summarize, FY26 was a year of meaningful financial progress, growth across revenue, EBITDA, and PAT, a successful IPO, and a materially stronger balance sheet. With a fully funded expansion plan, a debt-light structure and a category that offers a long runway, we are entering FY27 in a position of strength.

Thank you everyone for your time. We are happy to take your questions now.

Moderator:

Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Darshil Pandya from Finterest Capital. Please go ahead.

Darshil Pandya:

Yes, hello madam, and thank you so much for running us through such a great data. Madam, my first question is with regards to the opportunity size that you have shown. We understand that a lot of people are affected, but at the same moment of time, maybe I would want your comment on this. This looks like this treatment is quite expensive as we compare to Tier 2, Tier 3 cities where affordability is also a question.

So, what will happen in this case, where since I am not sure if this is covered in insurance or not, but what happens to affordability, as you said it has to be accessible, just wanted your

views on it?

Manika Khanna:

Yes, excellent question, Darshil. So basically, the base price at Gaudium for a cycle is INR 2 lakhs. And why we do not dilute this, you must have seen some small-time mushroom clinics putting ads behind an auto saying get treatment for INR 60,000 or INR 70,000. But what is important is that we are creating human life. So, it's not a fridge or a television. So, we can't mess with life. So, keeping the basic standards, so that the baby we create is healthy, we have to keep a base price of INR 2 lakhs.

But having said this, your question is very valid that when we go to Tier 2, Tier 3, even village level, infertility is there. You'll be surprised to know 8% to 10% infertility is there in villages also. To make it more accessible, so Gaudium has just formalized a Pan-India tie-up with HDFC Bank to help patients with financing. We also have two or three NBFCs whose representatives are available at our hubs and in addition to this, because here paperwork and all that happens. So, in addition to this, now Gaudium is taking a big leap, and you will be hearing very soon, we started with a credit scheme and it's going very well.

And we are going to extend this facility to our patients so that they can do their payment(patient) in a limited phased manner. So, we retrieve the egg in time at basic price and whatever our cost is, gets covered. And retrieve the egg, make the embryo, and freeze it. So, the aging of the egg stops.

What is most important to note in infertility is the egg gets older as age increases. When someone does not have money, that poor person will collect money for 3 years, by then the egg will age, so we will retrieve the egg and freeze it and keep it. And we will slowly give them a time of five-six months to complete the payment, and as soon as the payment is complete, then we will transfer their embryos.

So, the embryo safely stays with us. And this is supported by the new law because it says that, now you can't take genetic material from one clinic to the other like this, and we are allowed to keep genetic material up to 10 years. So, we are also taking this initiative to make it more and more affordable. As we become more accessible, we will increase affordability. So that will be also for the good of the people and great for business, because the numbers are only going to increase.

Darshil Pandya:

Understood. Well, thank you for this answer. My second question would be on the hubs that we today have. I guess around 7 hubs and 28 spokes today we have. Madam, this number will be, if I'm right, it will be 17 hubs by this year end?

Manika Khanna:

Yes, you're absolutely right. Yes.

Darshil Pandya:

Okay. And just to understand, what is the kind of, not just revenue, but what will we expect from this new addition that comes up? How will this help us in the business?

Manika Khanna:

Yes. So, what happens is that the additions will keep happening every quarter, like right now

in this quarter we are working on three hubs. So, what happens is that a hub is worked on, then it opens, its certification happens, and then footfall starts. So opening 10 hubs, meaning some will open in the first quarter, some in the second, some in the third, and some in the last quarter.

but we still see that even if we put very practical estimations on them opening, becoming operational, and getting patient footfalls, I would say that we are very confident that we will be able to maintain the high growth both in terms of profit and revenue as we have done historically, because our principle of being asset-light will remain the same. So, we are very confident that we will achieve this growth.

Darshil Pandya: Okay. Last question. Madam as you said that our cost is one-fifth in India as compared to the whole world. Madam, what is the percentage of these international clients that we would have, in the last year, and how are we planning to increase this in coming years?

Manika Khanna: Okay. So, I think approximately 25% to 30% is our contribution currently. And we are very aggressively working on this because this is good because obviously their packages are quite well decently priced. Secondly, it brings in more dollar influx inside the country. So, and for this, we are opening three international spokes in this quarter itself: Paris, Nigeria, and Sydney.

Manika Khanna: Yes, these are footfall based, from where we are already getting footfall. So yes, this is going to be a continuous exercise.

Moderator: The next question is from the line of Abhijeet Porwal from DR Choksey. Please go ahead.

Abhijeet Porwal: Yes. Thank you for taking my question, and congratulations on the good numbers. So firstly, I want to ask about the number of cycles performed this quarter and for the full year. Can you please update upon that?

Manika Khanna: Okay, so we use the matrix of cycles as the number of fresh pickups, because that is closest to unique patients. Because in IVF, people report differently. Some report pickup plus embryo transfer; some report embryo transfers. We have taken the global standard because there are only three other listed companies across the world in IVF, so they all report by the number of pickups done, meaning how many egg retrievals happen. So, we have already done roughly 2,300 pickups this year. And yes, with our aggressive growth plan, this number is bound to increase.

Abhijeet Porwal: Okay. So, like for the FY26, can you please quantify?

Rakesh Sharma: 2,255 pickups we have done, to be precise.

Manika Khanna: You are asking for the future year?

Abhijeet Porwal: No, no, for FY26, I am saying.

- Rakesh Sharma:** FY26, 2,255 cycles pickups we have done.
- Manika Khanna:** 2,255 ovum pickups, OPU's we call them.
- Abhijeet Porwal:** Okay. And ma'am, like the number of total embryologists for treatment are I guess five, right?
- Manika Khanna:** Yes.
- Abhijeet Porwal:** And so, the per embryologist cycle is, as per my understanding is high, am I correct or is this the industry standard? Like a good amount of higher degree of treatments is there?
- Manika Khanna:** Yes, yes. We are very well positioned as per global standards. Because what happens is that we are hardworking people. We optimize our results very well. And it works to an average of 120 pickups per embryologist, which is very easily manageable.
- And as the strength of the number of hubs increases, the number of embryologists and number of clinicians are going to increase. And of course, with the addition of the AI tools, SiD and ERICA, they act like AI assistants to the embryologist. So, we are kind of setting very high standards in embryology.
- So reduced time per patient, because the AI assistance make the decisions in milliseconds, which the embryologist's human eye will take longer. So, with the addition of these AI tools that we just launched, the time taken per case is also going to reduce. And the beauty in IVF is the faster you do the procedure, the higher is going to be the result. Why? Because you have kept embryo, egg and sperm that much time less outside incubator so less you expose to atmosphere and faster the embryologist works, the higher is the result. So, these AI tools are also going to significantly add to this.
- Moderator:** The next question is from the line of Shravani Salvi from Neomile Capital. Please go ahead.
- Shravani Salvi:** Hi, Ma'am. Congratulations on a good set of numbers. I have two questions. First one is, where do you see our ROCE in the next 3 to 4 years once all the hubs are completely operational?
- Manika Khanna:** Okay. I'll pass this to Rakesh.
- Rakesh Sharma:** Yes. See, the return on capital employed will be roughly 35% to 40% expected in the next 3 to 5 years. Then we will be fully functional.
- Shravani Salvi:** Okay. Sure. Next question is in terms of the international patients. What will be the current contribution of international patients as compared to the overall patient volume and growth? And which geographies are driving the highest inflows currently?
- Manika Khanna:** Yes. So, I think you missed the previous answer. We've already said that about 25% to 30% contribution is coming as of now. It is only bound to increase as we are putting three international spokes very soon in this quarter itself. And we will continue this exercise

throughout the year. And also, so we are targeting growth from medical tourism very aggressively.

Shravani Salvi: Okay. Could you give a number on the segment or the growth for the segment?

Manika Khanna: See, what I can comment is there is a huge potential because the cost of treatment is like many folds high there. The geographies we are getting patients from are Middle East, the entire African continent because it is highly underserved in this facility. Then we are getting NRIs from UK, we are also getting Australia, US, we are also getting native Americans, black Americans from all these countries. So, there is a decent mix of 30 plus countries from where we are getting these patients and we will continue growing as we have grown historically.

Shravani Salvi: Okay. Thank you ma'am.

Manika Khanna: You're welcome.

Moderator: Thank you. The next question is from the line of Tushar Bajaj from Pareto Capital. Please go ahead.

Tushar Bajaj: Hi ma'am, this is Tushar from Pareto Capital. So, I had a question on the trade receivable. So, we could see the revenue has increased from INR 70 crore in FY25 to INR 104 crore in March '26. Comparatively, the debtors have also increased in line. So, we just wanted to understand your brief overview on like how are we planning to manage the trade receivable and why is it high as compared to other peers in the industry?

Manika Khanna: Okay, so good question, Tushar. So basically, as I just spoke about affordability, so we said that we are trying to make it more and more affordable for the patients. That is what I meant, that we are making sure that a certain cost, that comes in our basic procedure for consumables and disposables to do a case is covered, so we initially take a certain sum of money from the patient and then we form their embryos and keep them.

So that revenue recognition happens stage-wise. So, what happens is that some amount of trade receivables show balance, but those embryos are safely kept with us. Now its reasons are split into, for some patients due to medical reasons the embryo transfer is delayed. For example, if suppose they are usually diabetic, so right now to control diabetes, we have taken out the egg on time so that its age doesn't increase, but in preparing the uterus, so that the patient's diabetes is controlled, sometimes it can even take 5 to 6 months.

So, we transfer then so that the result is good, and this is where internationally the world is going towards the freeze-all egg policy. This is called freeze-all, that you have frozen everything and delayed the transfer.

And the second category of patients is for financial reasons, that poor person has to take treatment from Gaudium because they know the result will come here, but they don't have that much money at once. So, by giving some money they can take global level treatment,

but we give them some time to make the payment to us, so that is why the receivables come.

And there is zero bad debt till now. And we wish that we can increase this affordability more and more, and to collect the receivables faster, now we have also taken a little initiative that the patients who do not come for transfer for more than a year, within one year, they will have to pay INR 40,000 extra. So, it will be a win-win situation.

Tushar Bajaj: As a finance cost, right? The INR 40,000 which you are charging extra.

Manika Khanna: Yes.

Tushar Bajaj: So just wanted to get your view, like what is the average tenure a customer takes to pay, meaning whoever has taken it on loan, your services, right? What is the average tenure? Like does it usually take more than a year or is it like 6 months to 2 years? What is a range-bound period?

Manika Khanna: So roughly, right now the exact number coming out is around 150 average days. And as I mentioned, we are taking measures to speed this up. One, we have applied an additional cost. Second, our calling team is also calling and educating the patients that the earlier they get it done, the better it will be.

And the third step we have taken is that now we have doubled the initial payment that we used to take INR 40,000 earlier. And now the new patients who come forward will have to pay INR 80,000 first so that the receivable that remains will be less moving forward. So, we have taken these three measures, and we are very confident you will see at the end of this quarter itself that a very significant difference has come, trade receivables will reduce significantly.

Tushar Bajaj: Okay, okay. One last question. Do we have a bifurcation like what amounts to the trade receivable, as in what amount is related to those financing thing? And what is related to those patients who have already done their first whatever freezing thing, and they will be getting the IVF done later?

Manika Khanna: It is almost the same for both.

Moderator: The next question is from the line of Deepali Sali from Punyavrat Capital. Please go ahead.

Deepali Sali: Yes, very good numbers ma'am and thank you for a very good service that you are giving to the world actually.

Manika Khanna: Thank you.

Deepali Sali: Yes, one question from my side is that there are many IVF players present in India as well as overall world, right? In India there are mainly two, three players like Indira, Nova, Oasis. And Indira is claiming that the success rate is around 75% to 76%. And according to your document, your success rate is 58%.

So, as we all know that getting children is an emotional subject, but people are ready to pay. You are positioning yourself as a premium IVF player, right? Then looking at the success rate, why will people come to you rather than going to Indira?

Manika Khanna:

Deepali, I will just say this that our success rate is 58% at the first cycle, which is at par with global standards. I would not like to comment on how people write success rate percentages, you compare with global. ASRM, which is the American Society for Reproductive Medicine, considers 40% to be a very good success rate.

So, 58% at one attempt is a very high success rate and is comparable to global best and also Gaudium has an 85% cumulative pregnancy rate. That means that 58% of our patients get pregnant in the first attempt, 80% in the second attempt, 85% in three attempts. Which means that if 10 couples come here to Gaudium, then there is an 8.5 chance that they will definitely leave pregnant.

This is comparable to some of the best players in the world like Boston IVF. And also I am very happy to share with you people that right now even if the data is small because we launched it only on the 1st of April, but the day before yesterday itself we had a small celebration here because with the addition of SiD and ERICA, in the early results we are seeing an almost 8% increase in result at the first attempt. So that is why we see that, you know, clinical excellence is our biggest tool.

Deepali Sali:

Okay, thank you so much for the answer, ma'am. And one more question from my side. I have seen that there is tax litigation against the company as well as the promoters. And two days before you have given some notification that some relaxation has been given. But this will be an overhang always. So, what are your views on this?

Manika Khanna:

Yes, so everything is at the appeal stage. And we are very confident. I would just say this.

Deepali Sali:

Okay. One more question. So, it took you around 16 years to establish seven centers, like seven hubs. And you are claiming that you are going to establish 19 in just 3 years. So how is this process going to come forward?

Manika Khanna:

Yes, excellent question. So, as I said, 2009 to 2018 was the journey of a doctor seeking excellence. We have reached the 85% cumulative pregnancy rate. We are in a completely SOP-driven, creating an SOP-driven organization in something as delicate as IVF, globally very few players have been able to do that, that whether we are sitting anywhere, whether I am in Delhi and the same result comes in Kashmir, the same result comes in Mumbai, the same result comes in Bengaluru, within plus minus 1% of each other.

To achieve this, it was a journey. But now that the model is all set and proven that expansion through 7 hubs and 28 spokes, we have still maintained the same clinical excellence. So now we are at a stage that we just needed capital to rapidly expand. So, we have a big organizational structure.

We have a Chief Operating Officer, a Chief Finance Officer, a Chief Administrative Officer, a Clinical National Clinical Head, a National Clinical Embryologist, whose reporting lines are defined. We have created such a solid structure that now the building's basement is ready, it won't take time to build the floors above.

Moderator:

Thank you. The next question is from the line of Puneet Kamra from Trustworthy Wealth Partners. Please go ahead.

Puneet Kamra:

Well, thank you so much ma'am for a great overview on the business. Really, really helpful to understand this. And congrats on the great set of numbers. I have a couple of questions. One is, most of the questions have been answered by other guys. So, if you could just tell us that how is the pricing based? Is it based on cycles or is it based on – OPU's?

Manika Khanna:

Yes, so generally what happens is that it is one complete cycle in the sense that there will be stimulation through injections, then egg pickup will happen, embryo formation will happen, and the first embryo transfer will happen. So that 2 lakhs is one complete cycle which includes one pickup and one embryo transfer.

Now if the patient gets pregnant after that, then perfect. If they do not get pregnant, then Gaudium's biggest strength is that we very rarely do a second stimulation. We have been pioneers in egg freezing and embryo freezing. We were amongst the first ones in the country to start. So, our freezing technology is so good that extra eggs from the first cycle, freeze them. And we can keep them for up to 10 years.

So, what happens is the patient has to undergo a thaw cycle. Thaw is very easy for the patient because injections won't be given again, will only undergo an embryo transfer. So, for that they pay separately, which is roughly 50% of the cost of the first cycle. And it's a win-win for us also because our expense in that is hardly anything. It's a very small expense and so the margin in that cycle is even higher.

And for the patient it's a win-win because they get a second cycle quickly without getting injections. And the rest of the packages vary with problems too. Like some of our patients have higher packages also, like multiple failed IVF cases, they are considered difficult cases. In those cases, there are many advanced technologies, laser hatching, SiD, ERICA, when these are used, the package costs go up.

And sometimes patients say that give us a multiple cycle package, some packages are of INR 20 lakhs, INR 25 lakhs also, where the patient has a good paying capacity and he says you take a lump sum from me and whenever I conceive, but you just give me these many attempts in a package. So yes, this is all a variety of patients that we deal with.

Puneet Kamra:

Right, got it. That's helpful. My second question is, based on my understanding, I understand that players like Indira are quite big and they have private equity capital in the business and given the fact that there is so much of gap in terms of demand and supply. So, would Gaudium be seeking private equity capital at a certain point in time? I mean I know you're listed, but

then there's always a space for private investments in public equities.

Manika Khanna: Yes, you're right. But we have received such a tremendous response from the market, we are very happy with our IPO, how it's progressing. And I am sure with the IPO proceeds and with our internal accruals we will be able to grow very fast. So as of now at this juncture, we are not looking at PEs. Because we want our vision to not to be diluted under any pressure.

We do not want our clinical excellence to become secondary to anything. Because Gaudium believes that as many women want to become mothers, we should be able to fulfil their dreams.

Moderator: Thank you. The next question is from the line of Anuj Kashyap from A3 Capital. Please go ahead.

Anuj Kashyap: Hello, hi. Good afternoon.

Anuj Kashyap: Congratulations, ma'am, for the great set of numbers. So, ma'am, as in the presentation and as told by you that we are an SOP-driven organization. So, ma'am, I just wanted to know how we make sure that Ms. Khanna is present at every hub or there is some differentiation while you select the people or like the SOPs are structured in such a way that the efficiency which Ms. Khanna delivers in Delhi and that gets delivered elsewhere also. So, what is the, like it's the people or it's the process?

Manika Khanna: Great, great question. I am glad you asked this. So, SOPs, we started making them around 2014-'15 itself. And you will be surprised to know our SOPs are right from the clinical embryology down to the housekeeping. Even what material the mop will be, what solution will be used, we decide that too. We design our labs and OT.

So, whatever I learned with experience, I have translated them into SOPs. I am out of clinical operations since 2019. So, my role is mainly to ensure that my SOPs are implemented everywhere. So, I am also making surprise visits, ensuring that all the chain of command is being followed. My chief clinician is getting the reports from every clinician. My chief embryologist is getting the same.

Similarly, in the accounts department. Similarly, we are very, very compliance-conscious people, and that is why we are very happy after the regulatory has come in. So, you know, all this, as you rightly asked, is all process driven. So, whatever was the extract of that experience, has been put into the process.

And the most important thing is that at our place, before being posted to any new centre, the clinician, the embryologist and the chief nursing, they all undergo an in-house training, no matter how much experience they come with. They have to undergo a compulsory in-house training at Delhi headquarters.

Then our chief embryologist takes an exit exam, chief clinician takes an exit exam, chief

national nursing in charge takes an exit exam, before we place them to a particular centre. So that we are sure that when this person goes to Mumbai or Nagpur, they will deliver the same results that the headquarter is delivering.

And the proof of this is that all our centres are giving results within plus minus 1% of each other. And all the new centres that are opening, like for example next month a centre is opening, all their trainings are already under process or completed.

Anuj Kashyap: Okay, ma'am, Thank you. And ma'am, I wanted to ask you, you have very well said that all your experience you have put in a process and ma'am, what makes sure, like when we train someone, so there are some clauses or contracts, like they will work with us, they will not leave us, so that our copyrights or something doesn't get transferred or something?

Manika Khanna: Yes. So, our legal team ensures that there is a lot of paperwork is done when we are hiring these people. They cannot, you know, kind of share data. We do not allow individual private practice. Whoever joins Gaudium has to be exclusive. Our clinicians and embryologists, we sign with them for a minimum of three years. And secondly, because the good part is that my national chief embryologist has been with me for about 12 years out of my 16-year journey.

Anuj Kashyap: Ma'am, this is a good thing about an organization then the attrition rate will be low?

Manika Khanna: Yes, so what happens is that we not only give them professional growth, but we also give them annual increments. So, you know, when they are getting both increase in, they are growing financially and professionally because for a doctor professional satisfaction matters a lot. So, the kind of learning they receive from us and then they implement it.

So, because of all these reasons, luckily, people don't leave so soon. The meaning of the word Gaudium in Latin is joy and happiness. So, we ensure that it is not only transmitted to our patients, but also to our employees. And you will be happy to know that we are a very women empowered organization and 90% of our employees are women.

Anuj Kashyap: Okay, that's great ma'am. That's great. And ma'am, one last one ma'am. Like you said, the incentives for the service, I mean, incentives. Is it linked to the success rate or how it is?

Manika Khanna: No, no. It's an annual increment. The script is based on performance. And now, performance measurements are available everywhere. You know how many patients you were able to convert, how many...

Anuj Kashyap: And then, one last question, ma'am, just a small one Ma'am, as an entrepreneur, Ms. Khanna, I mean, I just want to read your brain yes, yes. A Like, five years down the line, can we see Gaudium only in terms of women's fertility or can we also cover all-together women's health? If we grow up and prosper, then why not bring all the gamut into Gaudium? Yes, just as a. Yes. I want to hear your thoughts Yes?

Manika Khanna: Yes, so see our company's name itself is Gaudium IVF and Women Health. So of course,

right now our focus is completely on IVF. But our long-term vision is completely to support all of women's health. And soon you will be hearing good things.

Anuj Kashyap: Thank you ma'am. May Gaudium be true to its name and bring joy to everyone. Thank you.

Manika Khanna: Thank you so much.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. With that, I now hand the conference over to Mr. Vishad Khanna for closing comments.

Vishad Khanna: Good afternoon, everyone. I just want to thank you all for joining us today. And thanks a lot for showing us so much trust throughout this process and through these months. We are in very high spirits moving into the coming quarters.

As our hub expansion will be rolling out and our AI-led embryology is embedding deeper and deeper into our daily clinical workflow. So just want to end this with a big thank you to our patients, our team and our shareholders, and please carry on the support. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, on behalf of Gaudium IVF and Women Health Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Manika Khanna: Thank you, everyone.

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